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June 16, 2025

Mehment Oz, MD
Administrator, Centers for Medicare and Medicaid Services
U.S. Department of Health and Human Services
200 Independence Ave. SW
Washington, DC 20201

Submitted electronically via Regulations.gov.

RE: Request for Information; Health Technology Ecosystem

Dear Administrator Oz:

Dexcom, Inc. is submitting this letter in response to the Request for Information; Health Technology Ecosystem (RFI) published in the *Federal Register* by the Centers for Medicare and Medicaid Services (CMS) on May 16, 2025.[1]

Founded in 1999, Dexcom, Inc. is the market leader in transforming diabetes care and management by providing superior continuous glucose monitoring (CGM) technology to help patients and healthcare professionals better manage diabetes. We are also the first to provide individuals the option of using biosensing technology to track their glucose level and its impact on metabolic health. Since our inception, we have focused on better outcomes for patients, caregivers, and clinicians by delivering solutions that are best in class—while empowering our community to take control of their health. In 2017, we obtained first-time Medicare coverage for a CGM system. Dexcom continues to lead the industry, recently launching the first generative AI platform in glucose biosensing into our over-the-counter CGM, Stelo. This feature helps users contextualize their health information and make proactive, informed lifestyle decisions. Dexcom appreciates the opportunity to comment in response to this RFI. Our comments below are arranged with headings reflecting the numbering of the specific questions raised in the RFI.

Patient Needs: PC-1

In this question CMS asks what health management or care navigation apps would help patients understand and manage their health needs, or those of their loved ones.

[1] 90 FR 21034.

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Dexcom CGM's companion app allows patients to see their glucose data across time and share that information with family members or caregivers. This critical information permits patients to respond to activities, food choices or medications as they impact their glucose level. The evidence supporting the use of CGM is overwhelmingly positive. For this reason, the American Diabetes Association has recommended its use by anyone managing diabetes, regardless of whether they use insulin or not.[2] In an industry first, data from Dexcom CGMs can be incorporated directly into patient medical records so that providers can easily personalize treatment plans accordingly.

In recognition of the value of this technology, we recommend CMS consider providing educational materials to beneficiaries with diabetes concerning this important treatment option. CMS should also encourage providers to incorporate CGM data into their medical records systems. As noted earlier, Dexcom's products permit users to share their information with others, including family members or caregivers. Their ability to follow the patient's condition makes it possible for them to support the patient in their diabetic journey and to intervene when acute events occur that may require urgent response. This capacity to share and follow a patient's glucose levels is particularly helpful for patients who live in long-term care facilities where they may not have the physical or cognitive capacity to respond to acute events. We suggest that CMS encourage those facilities to set up mechanisms for staff to track patient CGM data to help reduce the number of severe dysglycemic events and facilitate better care management in general.

We note that in late February of this year, a coalition of diabetes providers and stakeholders submitted a formal request to CMS' Durable Medical Equipment Medicare Administrative Contractors (DME MACs) requesting that coverage criteria for CGMs be updated to reflect the most current clinical research. We recommend CMS encourage the DME MACs to move expeditiously with regard to that request so that this important tool can be made available to patients whose conditions can be demonstrably improved with it.

Patient Needs: PC-5

In this question, CMS asks what can be done to encourage patient and caregiver interest in digital health products. The question further asks what role, if any, CMS should have in reviewing or approving digital health products.

[2] American Diabetes Association Professional Practice Committee; 7. Diabetes Technology: Standards of Care in Diabetes—2025. *Diabetes Care* 1 January 2025; 48 (Supplement_1): S146–S166. https://doi.org/10.2337/dc25-S007

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While the terms "reviewing" and "approving" carry specific regulatory meaning defined by the FDA, CMS might explore a distinct and complementary role in evaluating and endorsing digital health products.

By extension, CMS could certify that digital health technologies are interoperable across provider EHR and plan claims or provider network systems for use beyond a single plan or provider. To the extent that CMS begins to certify interoperability, or utility of specific digital health products, we recommend incorporation of CGM data into EHR systems as one criterion for such certification.

Dexcom continues to believe that digital health innovation is a critical element of healthcare for patients, caregivers and providers alike. Connected digital solutions enhance continuity of care from healthcare facilities to home and help optimize effectiveness of a treatment paradigm as part of the day-to-day care of patients and their caregivers. CMS can continue to evaluate coverage of digital health innovations that help improve communication between patients and caregivers, enhance their engagement in treatment adherence, and help reduce the need for costly procedures or medical visits without compromising patient outcomes.

Patient Needs: PC-7

In this question, CMS asks about the best means of collecting real-world data on digital health products' impact on health outcomes and related costs once they are released into the market.

Over the years, CMS has increased access to Medicare claims data to facilitate real world evidence development by qualified entities. While qualified entities are required to use Medicare data to create public provider performance reports, they are also permitted to conduct non-public analyses and to provide or sell those analyses to authorized users. Currently, qualified entities are not permitted to provide non-public analysis of Medicare claims to biopharmaceutical entities, making it essentially impossible for those entities to conduct retrospective, real world analysis of Medicare data. National data registries in other countries have proven invaluable in facilitating such research. For example, the Swedeheart registry is one of the most comprehensive and successful models of a national quality registry. Swedeheart collects data on patients with acute coronary syndromes and heart failure. The registry has been instrumental in improving cardiac care outcomes in Sweden and has contributed to numerous research publications that have influenced international cardiology practice. The registry produces publicly available reports and data for research. To facilitate similar efforts in the US, to the extent permitted by law, Dexcom recommends that CMS consider how applicable regulations might be modified to facilitate the creation of real-world evidence using Medicare's fee-for-service data.

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Ecosystem: TD-1

In this question, CMS asks what short- and longer-term steps the agency could take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers,

As described earlier in these comments, Dexcom recommends that CMS encourage plans, providers, and entities such as Accountable Care Organizations, to incorporate the use of CGM data into their EHR systems. This step would facilitate more personalized care for Medicare beneficiaries.

We also note the existence of algorithms that take data provided by CGMs and use it to drive the automated delivery of insulin via insulin pumps. These algorithms have been recognized by the FDA as a unique device type – the "Interoperable Automated Glycemic Controller."[3] The clinical effectiveness of automated insulin delivery systems, driven by interoperable controllers is so significant that the American Diabetes Association strongly recommends them for patients who require daily insulin.[4]

The development of these algorithms requires significant resources as well as on-going maintenance to ensure compatibility with a host of smart devices and upgrades in pump and CGM technology. Unfortunately, to date, Medicare has not provided coverage and reimbursement for interoperable controllers. Dexcom strongly believes that existing coverage and reimbursement pathways for DME supplies could be used to provide such coverage, and we encourage CMS to work with the developers of these interoperable controllers to establish appropriate coverage.

Another step that CMS could take to encourage the use of these very effective automated insulin delivery systems is to update NCD 280.14 to reflect the most current evidence. In 2022, a coalition of diabetes providers and stakeholders submitted a formal NCD reconsideration request, asking that CMS update Medicare's insulin pump coverage criteria. The policy was last updated more than 20 years ago and no longer reflects current clinical evidence. In fact, the policy prevents many people with type 1 diabetes and all people with type 2 diabetes from accessing these vital systems, even though they are considered standard of care and their use is backed by a great deal of clinical evidence. We therefore recommend that CMS prioritize the modernization of this important coverage policy.

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[3] See: https://www.accessdata.fda.gov/scripts/cdrh/cfdocs/cfpcd/classification.cfm?id=714

[4] American Diabetes Association Professional Practice Committee; 7. Diabetes Technology: Standards of Care in Diabetes—2025. *Diabetes Care* 1 January 2025; 48 (Supplement_1): S146–S166. https://doi.org/10.2337/dc25-S007

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Finally, we note that hospitalizations for hypoglycemia among Medicare beneficiaries are a recognized problem.[5] CGM use has been shown to significantly reduce the risk of hypoglycemia.[6] Consequently, we recommend that CMS develop and implement for use in the ambulatory setting, CGM-derived measures of patient hypoglycemia and hyperglycemia. In doing so, CMS would encourage providers to ensure their patients are using the best tools to prevent these serious complications.

Diabetes impacts approximately 32% of the Medicare population.[7] The overall cost of diabetes in the US in 2022 has been estimated at $413 billion.[8] Consequently, it is important that CMS promote actions by providers aimed at helping to control this chronic disease. For example, CMS could encourage providers to utilize the "Diabetes: Glycemic Status Assessment Greater Than 9%" quality measure in the MIPS program. This measure is an indicator of how well a provider's population is controlling their diabetes and the "glucose management indicator" (GMI) used in that measure is calculated using CGM data. Encouraging providers to report this measure will concurrently result in education for patients about the appropriate use of CGM, which will improve the management of their diabetes.

We thank CMS for the opportunity to provide comments in response to this RFI and look forward to seeing substantive proposals from the agency in furtherance of these goals.

Should you have any questions about our comments, please contact me directly at jesse.bushman@dexcom.com.

Sincerely,
/JSB/
Jesse S. Bushman, Director, US Policy

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[5] Lipska KJ, Ross JS, Wang Y, Inzucchi SE, Minges K, Karter AJ, Huang ES, Desai MM, Gill TM, Krumholz HM. National trends in US hospital admissions for hyperglycemia and hypoglycemia among Medicare beneficiaries, 1999 to 2011. JAMA Intern Med. 2014 Jul;174(7):1116-24. doi: 10.1001/jamainternmed.2014.1824. PMID: 24838229; PMCID: PMC4152370.

[6] Heinemann L, Freckmann G, Ehrmann D, Faber-Heinemann G, Guerra S, Waldenmaier D, Hermanns N. Real-time continuous glucose monitoring in adults with type 1 diabetes and impaired hypoglycaemia awareness or severe hypoglycaemia treated with multiple daily insulin injections (HypoDE): a multicentre, randomised controlled trial. Lancet. 2018 Apr 7;391(10128):1367-1377. doi: 10.1016/S0140-6736(18)30297-6. Epub 2018 Feb 16. PMID: 29459019.

[7] See: https://www.cdc.gov/mmwr/volumes/68/wr/pdfs/mm6843a2-H.pdf

[8] Emily D. Parker, Janice Lin, Troy Mahoney, Nwanneamaka Ume, Grace Yang, Robert A. Gabbay, Nuha A. ElSayed, Raveendhara R. Bannuru; Economic Costs of Diabetes in the U.S. in 2022. *Diabetes Care* 2 January 2024; 47 (1): 26–43. https://doi.org/10.2337/dci23-0085